Assessing Mental Status Including Risk for Substance Abuse

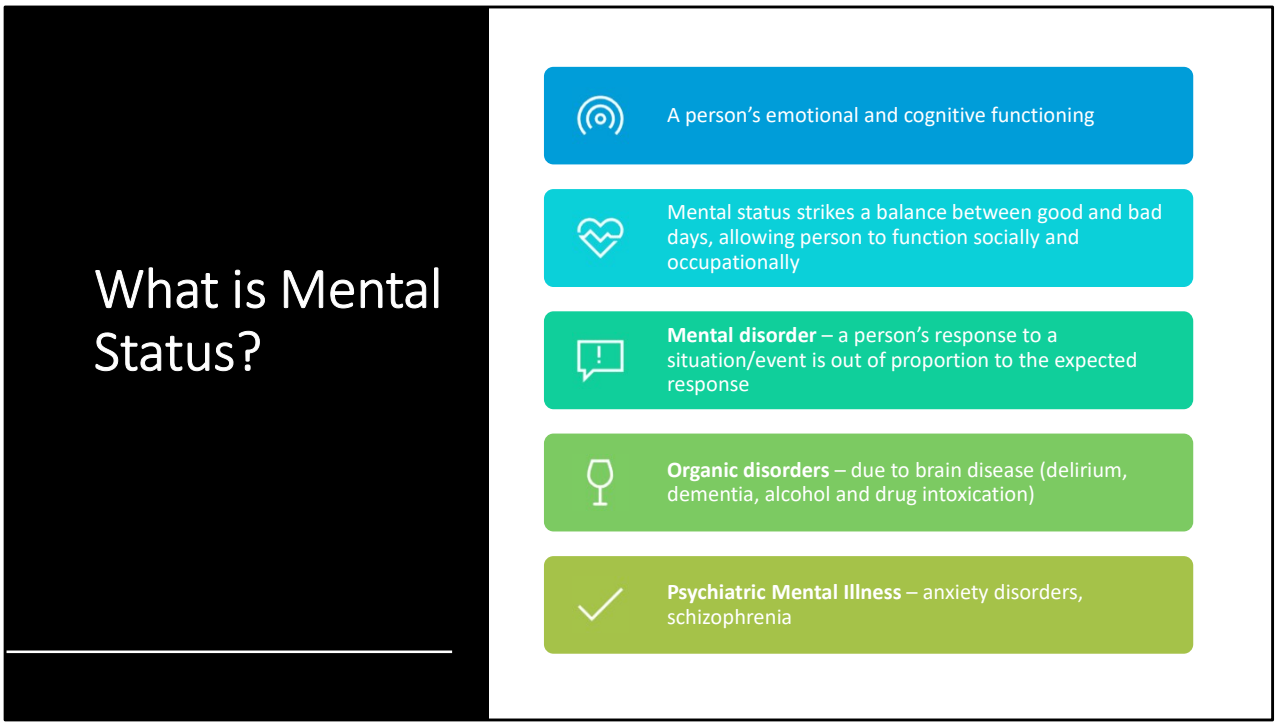
Weber: Chapter 6

NURS3120: Health Assessment and Promotion

Objectives

1. Discuss how both mental health and mental disorders affect mental status
2. Differentiate between normal and abnormal findings of a mental health status and substance use assessment
3. Analyze client's mental status and substance use risk assessment data to make valid clinical judgments





Mental status refers to a client's level of cognitive functioning (thinking, knowledge, and problem solving) and emotional functioning (feelings, mood, behaviors, and stability).

Mental health is an essential part of one's total health and is more than just the absence of mental disorders.
A healthy mental status is needed to think clearly, respond appropriately, and function effectively in all activities of daily living (ADLs). It is reflected in one's appearance, behaviors, speech, thought patterns, decisions, and ability to function in an effective manner in relationships in home, work, social, and recreational settings. One's mental health may vary from day to day depending on a variety of factors.



WHO (2022) Definition

"A state of mental well-being that enables people to cope with the stresses of life, realize their abilities, learn well and work well, and contribute to their community."

Factors Affecting Mental Health

- Biophysical factors
- Changes in the neurological system
- Economic and social factors
- Lifestyle considerations
- Exposure to violence
- Psychological factors
- Spiritual factors
- Cultural factors

As nurses we need to recognize that there are many factors that can affect a client's mental health.
Biophysical: Family history, TBI, chronic medical condition
Changes in the neurologic system: cerebral abnormalities that disturb the client's intellectual ability, communication ability, or emotional behaviors
Economic & social: financial concerns, lack of access to health care, loss of loved one
Lifestyle: sedentary lifestyle, poor nutrition, substance abuse
Exposure to violence: military, veterans, refugees from violent environments, violent household
Psychological factors: poor decision-making skills, low self-esteem, poor self control
Spiritual factors: existential crisis or cognitive dissonance
Cultural factors: cultural plays a role in perception of illness especially illnesses associated with mood and mental status

Mental Disorders

- Also called mental illness, psychiatric disorder, psychiatric illness, or psychological disorder
- Multiple factors can contribute:
  - Environmental
  - Psychological
  - Genetic
  - Chemical
  - Social

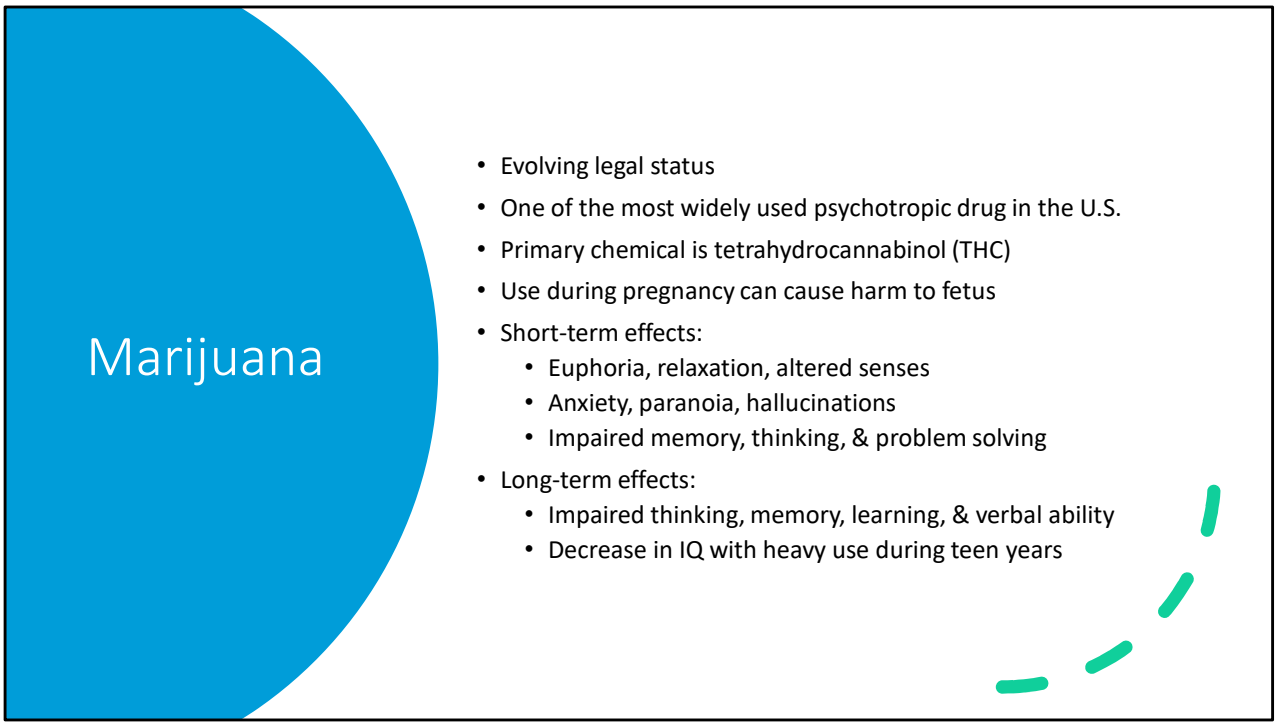


Mental disorder: “any condition characterized by cognitive and emotional disturbances, abnormal behaviors, impaired functioning, or any combination of these” Characterized by issues regarding mood, thoughts, and behavior.
Mental disorders often cause significant distress or impaired functioning or both.
It also effects other body systems. Example: Depression causes decreased appetite which will develop into nutritional deficiencies that affect the GI system.

Substance Use

- WHO (2023): “harmful or hazardous use of psychoactive substances, including alcohol and illicit drugs”
- Persistent condition that affects mental health
- Can arise from an underlying mental disorder
- Can lead to dependence syndrome
  - Cluster of behavioral, cognitive, and physiologic phenomena

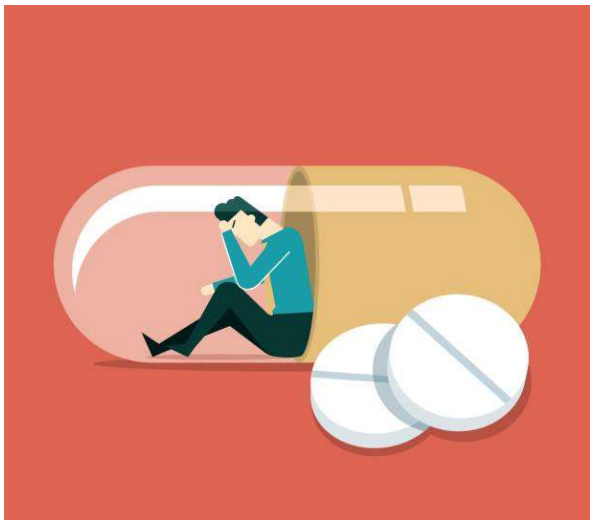
Dependance syndrome: The person has a strong desire to take the drug, difficulty controlling its use, and the need to continue its use despite harmful consequences. The use of the substance may become the person’s priority in life, resulting in avoidance of responsibilities and leading to a withdrawal from society and relationships.



Marijuana is a special case of drug use because the legal status of marijuana is evolving. Many states now allow medical use, and several states allow adult recreational use.

In pregnant people, marijuana use can cause harm to a fetus, resulting in low birth weight, or brain or behavioral problems later in life.

Long-term effects of marijuana use for those who begin before age 18 include impaired thinking, memory, learning, and verbal ability; heavy teen use continuing over years results in a significant decrease in IQ (by as much as 8 points), with no regaining of mental abilities after stopping use



Health Promotion & Disease Prevention: Substance Use Disorder

- Causes are complex
- Stopping is beyond the individual's control
- Chronic disease characterized by compulsive seeking & using one or more drugs of choice
- Dependency is accompanied by:
  - Brain changes in learning, judgment, decision making, stress, memory, & behavior

Healthy People 2023 goal: Reduce misuse of drugs and alcohol

Approximately 108,000 persons in the United States died from illicit or prescription drug-involved overdose in 2022. Most misused: Alcohol, tobacco, cocaine, heroin, hallucinogens, methamphetamine, and many other medically prescribed, over-the-counter, or illegal.

Substance Use Disorder

- Screening & early detection are crucial
- Risk factors:
  - Age at initiation, genetics, other mental health issues, sex assigned at birth, personality, family factors, community influence, societal elements
- Prevention programs aim to avert substance misuse & the associated adverse effects
- Education should be adjusted according to the developmental level of client

Screening for substance use disorders is a crucial component of early detection and intervention. Early identification can lead to timely referral to appropriate treatment, potentially preventing the exacerbation of the disorder and reducing the associated personal and societal costs.

Age at substance use initiation (higher likelihood of disorders if use is begun at a younger age)
Genetics
Other mental health issues (i.e., depression, anxiety, PTSD, etc.)
Sex assigned at birth (substance use can develop into dependency more quickly in people assigned female at birth)
Personality (risk-taking and thrill-seeking behavior)
Specific types of patterns of drug use (opioids, nicotine, and methamphetamine are more likely to develop into a substance use disorder; intravenous drug use is strongly associated with dependency)
Family factors (trauma, abuse, neglect, and parental use of substances increases risk)
Community influence (peer pressure, prevalence of drug use in school, and neighborhoods with high rates of violence and poverty)
Societal elements (stigmas and discrimination; laws and culture)

A few education points: Be aware of behavioral changes, keep medication in a locked cabinet, watch for slipping grades, be aware of emergency services

Collecting Subjective Data

- Biographical information
- Headaches?
- Mood swings/irritability?
- Insomnia?
- Homicidal or suicidal ideation?
- Hallucinations?
- History of mental health issues?
- Head injury/stroke?
- Serve in the armed forces?
- Family history?
- Medications?
- Alcohol/opioids/illicit drugs?
- Exposure to environmental toxins?

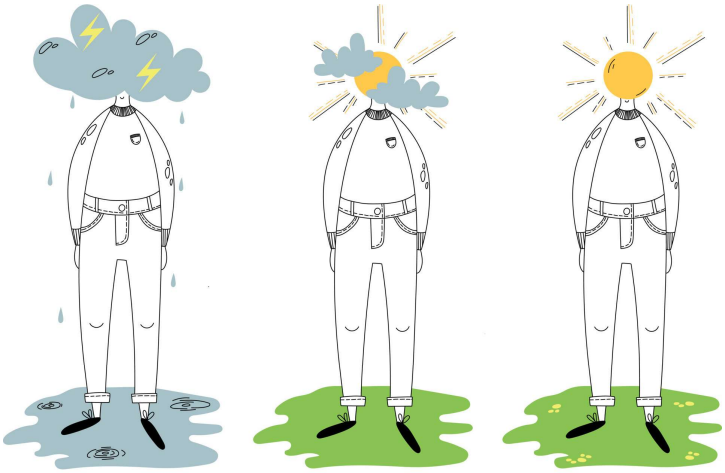
Asking these questions will provide a baseline of the client's level of consciousness
Some clients may have had a positive or a negative past experience with mental health care services or counseling that may influence their decisions to seek help in this area again. A past hospitalization for mental health may indicate a more serious problem than if the client received outpatient services.
Any head injury condition can affect the developmental level and the mental status of the client. Changes in behavior, communication patterns, and sleep habits as well as other physical changes, may occur with these conditions.
Posttraumatic syndrome may be seen in veterans who experienced traumatic conditions in military combat.
Some psychiatric disorders may have a genetic or familial connection such as anxiety, depression, bipolar disorders and/or schizophrenia, or Alzheimer disease. Effectiveness of past family treatments may give direction for future treatments for this client.
Use of these over-the-counter medications may alter one's level of consciousness, decreased response times, and cause changes in mood and temperament. Inappropriate use of any of these substances may indicate substance abuse problems.
Excessive drinking over an extend period can lead to certain types of cancer, liver damage, immune system disorders, and brain damage. It can also aggravate some conditions such as osteoporosis, diabetes, high blood pressure, and ulcers. Drinking in

some older adults may cause symptoms of forgetfulness or confusion, which could be mistaken for signs of Alzheimer disease. Cognition, mood, behavior may be noted with recreational drug use. Some recreational drugs increase risk taking and risky behaviors. Some people treat headaches or anxiety or other mental health disorders with recreational drugs.

Men >85yo are at greatest risk for suicide

Collecting Objective Data: Physical Examination

- Assesses the highest level of cerebral integration
- Perform at the beginning of the head-to-toe exam

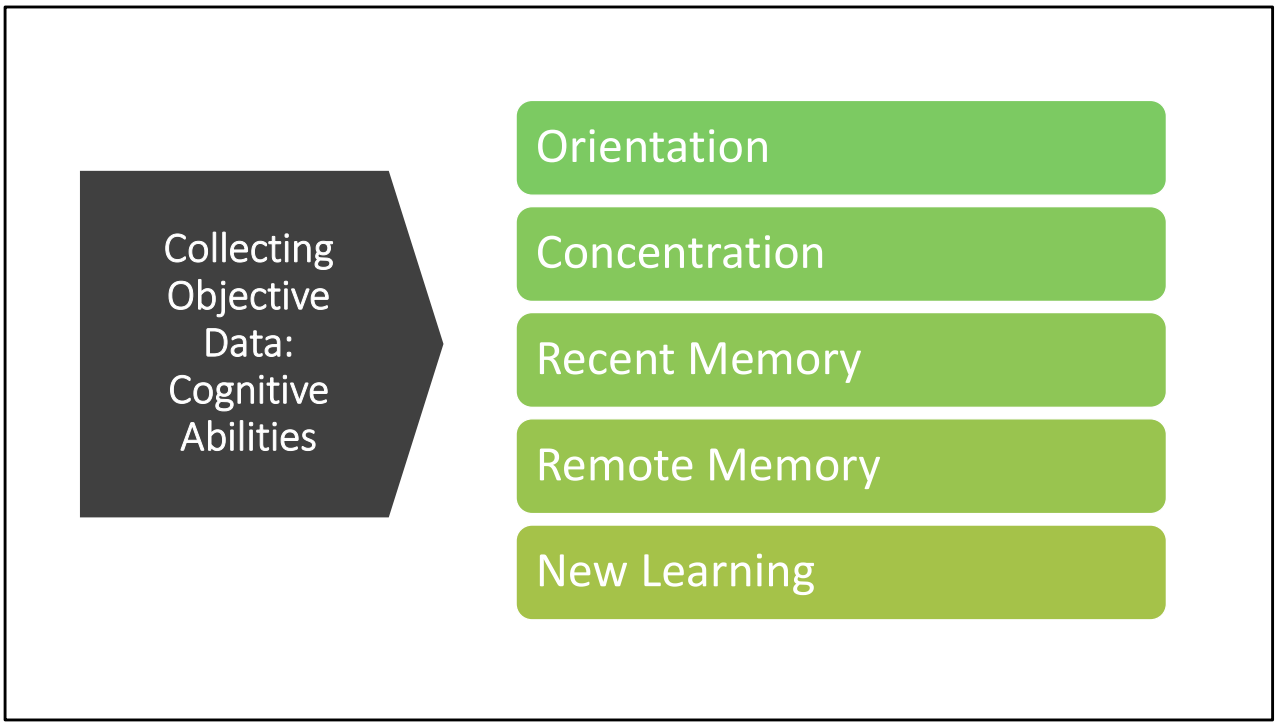


Of the neurologic assessments, the mental status examination assesses the highest level of cerebral integration. Many find assessing mental status at the very beginning of the head-to-toe examination advantageous, as it provides clues regarding the validity of the subjective information provided by the client throughout the examination.

Collecting Objective Data: Level of Consciousness & Mental Status

- Inspect/Observe
  - Assess LOC (level of consciousness)
  - Posture
  - Body movements
  - Dress/Grooming/Hygiene
  - Facial Expressions
  - Speech
  - Mood and Affect

LOC: Is the client awake, asleep, drowsy, unarousable?
Appearance: When you are "inspecting" your client notice their posture-do they sit up, slump over, withdrawn, Are their body movements rigid, jerky, do you notice tremors, How are they dressed? Do their clothes match, does it look like they just picked up clothes from the floor and put them on? So does your client smell, do they shower?
Behavior: there are five levels of consciousness that you need to know, especially if you are going to work in the hospital, this is something that is very important because something so small in a decrease level of consciousness throughout your shift be the only indicator that something is going on with your client. There is alert, lethargic, obtunded, stupor, and coma
Facial expression: does their facial expression match what they are saying? Or do you have a client says they are fine, but their face is stoic?
Speech: Are they slurring their speech?
Mood and affect: Does your client have a flat affect? Does anybody know what I mean by that? Are they angry? Withdrawn? Confused?



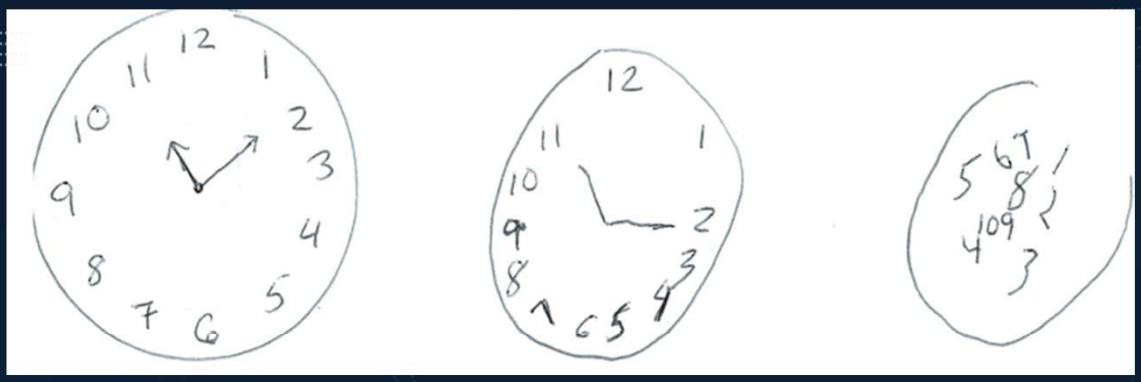
Cognitive Function: Orientation: Alert and Orientated to person, time, place and situation. When assessing orientation, remember that orientation to time is usually lost first and orientation to person is usually lost last. Reduced orientation may be seen in clients with organic brain disorders or psychiatric illness.

Concentration: Should be distraction free and able to follow directions. Inability to focus may be seen with anxiety, fatigue, attention deficit disorders, and impairment from alcohol or drugs.

Recent memory-what did you eat today? Inability to recall can be seen with delirium, dementia, depression, anxiety.

Remote memory- memory in the distant past or long-term memory- what high school did you go to? Inability to recall can be seen in clients with cerebral cortex disorders.

New learning-are they able to learn new things, like medications-new medications and what they are taking them for. Assess by having them repeat and recall four unrelated words at 5, 10, and 30 minutes. Inability to recall words can be seen in anxiety, depression, or dementia.



Collecting Objective Data: Cognitive Abilities

- Abstract reasoning
- Judgment
- Visual, perceptual, & constructional ability

Abstract reasoning: Ask client to compare/contrast objects. “How are an apple and an orange the same?” Inability to compare/contrast seen in schizophrenia, intellectual disability, delirium, dementia.

Judgment: Should be able to answer based on sound rationale. “What do you do if you have pain?” Impaired judgment may be seen in clients with organic brain syndrome, emotional disturbances, intellectual disability, schizophrenia.

Visual, perceptual, & constructional ability: Ask client to draw the face of a clock. Inability to correctly complete is seen with intellectual disability, dementia, parietal lobe dysfunction.



Alert: a client is alert and orientated to person, time, place, situation, A&O x4. Can a confused patient be alert? Yes of course, they might not answer the questions correctly, but they are fully aroused and aware of their environment.
Lethargy or lethargic is how you would document that a patient is slowed or sluggish? What type of client would you say presents with lethargy? Alcoholic, drug induced-morphine, client's just out of surgery that have anesthesia in their system.
Obtunded: Example a client that is aroused by gently shaking their arm and repeatedly calling their name, responds with one-word answers; follows simple commands; disorientated to time and place.
Stupor: Vigorous stimulation can be performed by the nurse by sternal rubbing hard. They open their eyes for a brief second then usually close their eyes again. Very minimal response from a client.
Coma: Those clients do not respond whether they are medically induced comas or something neurological happened to the patient. Does anybody know what I mean by a medically induced coma?

PLEASE KNOW THE DIFFERENCE BETWEEN THE LOC

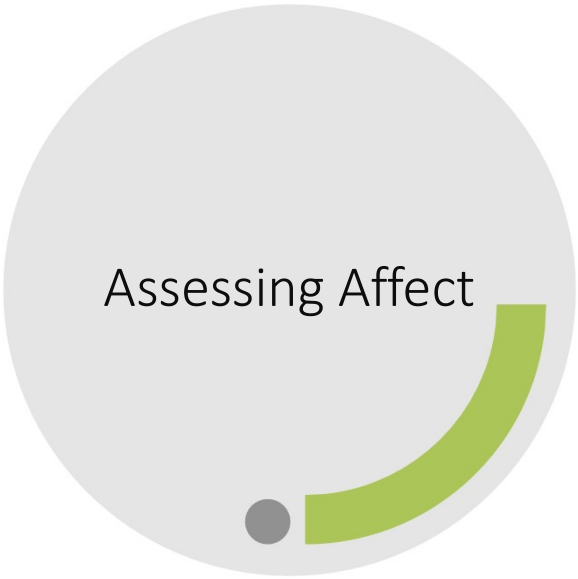
Behaviour	Response
 Eye Opening Response	4. Spontaneously 3. To speech 2. To pain 1. No response
 Verbal Response	5. Oriented to time, person and place 4. Confused, but able to answer Qs 3. Inappropriate words 2. Incomprehensible sounds 1. No response
 Motor Response	6. Obeys commands 5. Moves to localised pain 4. Flex to withdraw from pain 3. Abnormal flexion, decorticate posture 2. Abnormal extension, decerebrate posture 1. No response
Minor Brain Injury = 13-15; **Moderate Brain injury** = 9-12; **Severe Brain Injury** = 3-8 Record the GCS as a total and its tree sperate components: e.g., GCS 8/15: E3, V2, M3	

Assessment Tool: Glasgow Coma Scale

- **Eye Opening (E)**
  - **Score 1-4**
- **Verbal Response (V)**
  - **Score 1-5**
- **Motor Response (M)**
  - **Score 1-6**

Glasgow Coma Scale is used for clients who have experienced a traumatic brain injury: GSC score of less than 15 indicates some impairment in the level of consciousness. A score of 3 the lowest possible score indicates a deep coma. The GCS can not be used to assess a verbal score in intubated or aphasic clients however, it is still the most widely used scoring system for intensive care units.

- Euthymic
- Constricted
- Blunted
- Flat
- Labile
- Inappropriate



Affect is the expression of emotions or the individual's emotional state. A person may experience joy, anger, fear, and sadness. The person's affect will normally be expressed in behaviors that reflect the emotions.
Euthymic: Normal, steady, tranquil mental status (could be a temporary steady state between the up and downswing of affect seen in bipolar disorder)
Constricted: A mildly diminished range or intensity of emotional expression (associated with autism, schizophrenia, depression, PTSD, brain injury)
Blunted Markedly diminished emotional expression (associated with autism, schizophrenia, depression, PTSD, brain injury)
Flat: a severely reduced emotional expressiveness (depression/schizophrenia)
Labile: irregular and severe mood swings (bipolar, borderline personality disorder, overstimulation, stress or anxiety)
Inappropriate: emotional responses that are not in keeping with the situation or are incompatible with expressed thoughts or wishes (disorganized schizophrenia or with brain tumor/injury)

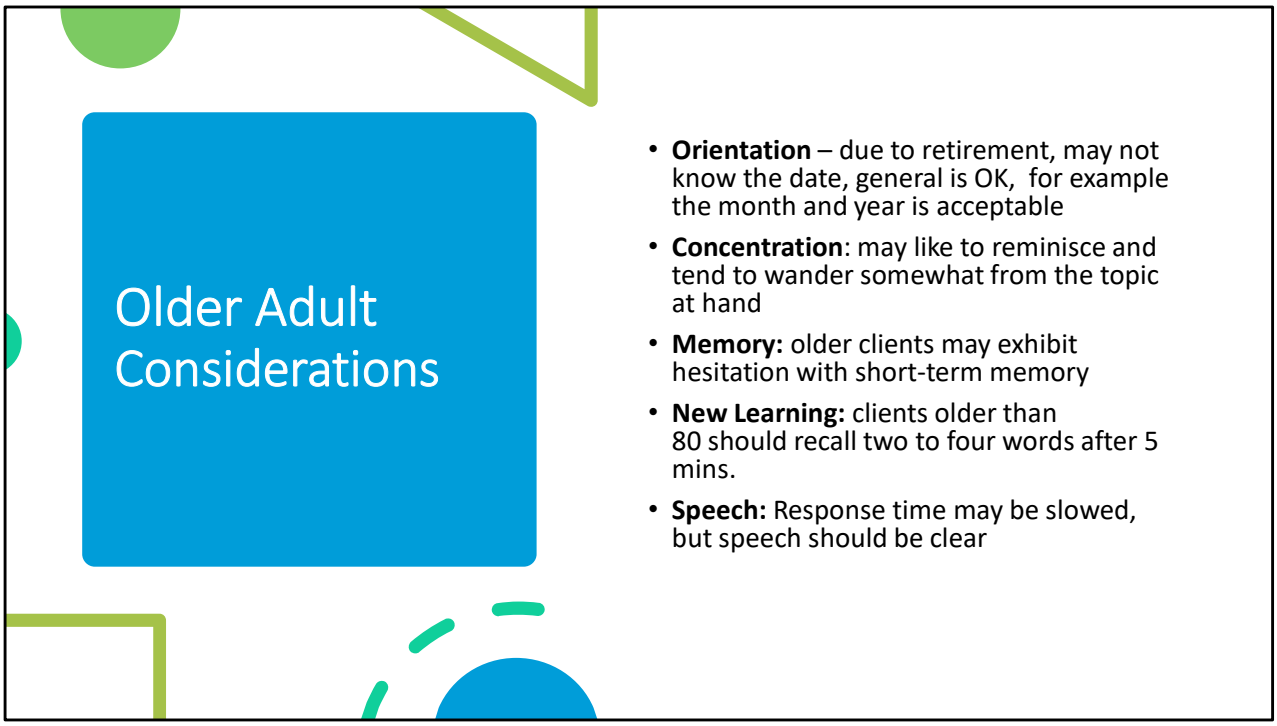
Older Adult Considerations

- **Assess vision & hearing first**
- May wear excess layers due to decreased metabolism and cold intolerance
- Older adulthood contains more potential for losses
- Chronic diseases such as heart failure, cancer, diabetes, and osteoporosis include fear of loss of life
- Older client's response and ability to process information may be slower, they remain alert and orientated



When assessing the mental status of an older client, be sure to check vision and hearing before assuming that the client has a mental status problem.

Grief and despair surrounding these losses can affect mental status and can result in disability, disorientation, or depression



May seem confused in a new setting like the hospital, but most know who and where they are and the current month and year.

Clients older than 80 years should recall two to four words after 5 minutes and possibly after 10 and 30 minutes with hints that prompt recall.

Alzheimer Disease vs. Delirium vs. Depression

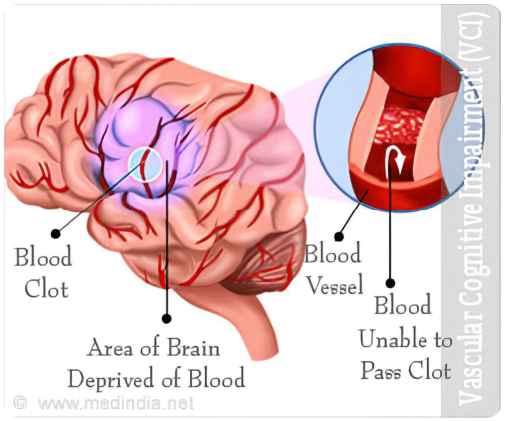
Alzheimer Disease: Onset-slow and insidious, progresses; Mood-depression is common; Speech-intact until late phase. Orientation- Early: confusion to familiar places and objects; Late: no recognizable people, places; Physical signs: no deficits early to loss of all voluntary activity late in disease; 75% of dementias

Delirium: Hours to weeks in duration; Acute onset; Impaired/fluctuating attention; Recent and immediate memory impaired; Fluctuates from lethargic to hypervigilant; Disorganized thinking; Occurs in up to 80% of hospitalized people, predominately older adults

Depression: Can last weeks to months or years; Often abrupt onset; Distractible but minimal impairment of attention; Islands of intact memory; Alert; Thinking intact though with themes of helplessness or self-depreciation

Alzheimer disease can be early (40-65 years) or late (65+ years) onset. Most common 85+ years.

Delirium can be caused by drug toxicity and interactions, acute disease, trauma, chronic disease exacerbation, fluid and electrolyte disorders, fever, UTI, dehydration…



Vascular Dementia

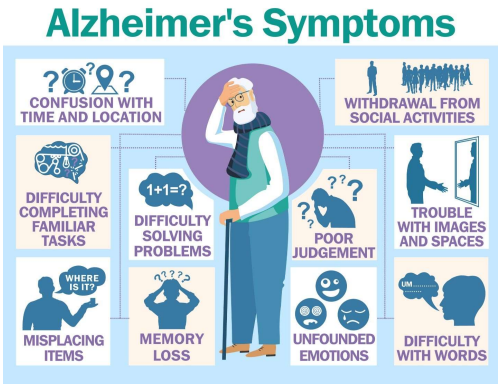
- Risks
  - Cardiovascular disease
  - Cerebrovascular disease
  - Hypertension
- Makes up 10-20% of dementias
- Often follows a stroke or TIA (Transient ischemic attack)

Predominantly in those assigned male at birth.
Labile mood/mood swings
May have speech deficit/aphasia depending on the location of lesion
Commonly exhibits motor deficits
May fluctuate between lucidity and complete disorientation
Impaired recent & remote memory
Hallucinations, paranoia
May fluctuate between high agitation, hyperactivity, restlessness, and lethargy

Heath Promotion & Disease Prevention: Dementia & Alzheimer Disease

- Dementia: A group of symptoms that cause problems with memory, thinking, and behavior
- Alzheimer disease: The most common cause of dementia
  - Gradual destruction of brain nerve cells & a shrinking brain
- Alzheimer disease was the 5th leading cause of death in individuals 65 and older in 2019

Healthy People 2030 goal: Improve health and quality of life for people with dementia, including Alzheimer Disease



Normal aging has common forms of decline that are often mistaken for dementia or resemble dementia. These include slower thinking, problem solving, learning, and recall; decreased attention and concentration; more distractedness; and need for hints to jog memory. It is important to differentiate dementia from common cognitive changes that occur with age.

Symptoms of Alzheimer Disease resemble general dementia symptoms but in the various stages may include loss of recent memory, depression, anxiety, personality changes, unpredictable quirks or behaviors, confusion, aggression, agitation, suspicion, wandering, trouble sleeping, inability to recognize family members, and problems with language, calculation, and abstract thinking. Inability to manage a budget is a common symptom. Gradually worsening ability to remember new information is a key symptom.

Dementia & Alzheimer Disease

- Screen using the Everyday Cognition questionnaire
- Risk factors:
  - Increasing age, especially 65 and older
  - Genetic predisposition & family history
  - Mild cognitive impairment
  - Hispanic ethnicity or African descent (Higher rates of vascular disease)
  - Smoking
  - Not maintaining healthy aging behaviors

- Education for protecting brain health:
  - Exercise
  - Education
  - Avoid head injury
  - Quit smoking
  - Healthy diet
  - Challenge the brain with activities like puzzles, brain teasers, reading, crosswords
  - Activities that require both physical and mental exertion like cooking, yard work, playing with pets

Everyday Cognition questionnaire:
Remembering the location of objects
Remembering the current date or day of the week
Communicating thoughts in a conversation
Understanding spoken directions or instructions
Reading a map and helping with directions when someone else is driving
Finding the way around a familiar house or building
Anticipating weather changes and planning accordingly
Thinking ahead
Keeping living and work spaces organized
Balancing the checkbook/account without error
Doing two things at once (e.g., cooking and talking on the phone at the same time)

Questions ??


Please remember the real me
when I cannot remember you.
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